INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Sorafenib: differentiated thyroid cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00294
Version 4

> Twice daily, continuously

Treatment stages

01 **Continuous oral treatment**
Twice daily, continuously

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	4
03	Symptoms and supportive care	9
04	Your record and contact card	14
05	Sources and further information	17

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

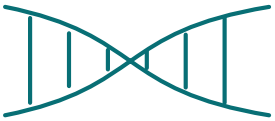
How the medicine works

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

KINASE INHIBITOR

Sorafenib

Oral targeted treatment



Blocks growth and blood-vessel signalling

Sorafenib blocks several enzymes involved in signals that help cancer cells grow and new blood vessels form. It can also affect healthy blood vessels, skin and thyroid hormone control.

Growth signals blocked → Blood-vessel signals reduced → Cancer growth slowed

What to watch for

Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Research supports a treatment discussion; it cannot predict one person’s outcome.

DECISION, a randomised placebo-controlled study in radioactive-iodine-refractory differentiated thyroid cancer, reported median progression-free survival of 10.8 versus 5.8 months (hazard ratio 0.587; 95% CI 0.454–0.758). The cited overall-survival analysis did not show a statistically significant difference. Progression-free survival measures time without progression or death, not a predicted lifespan.

CHAPTER 02

Your schedule, tests and appointments

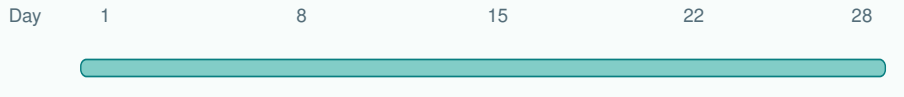
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Sorafenib: differentiated thyroid cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TWICE DAILY, CONTINUOUSLY

Continuous oral treatment

Continue while helping and tolerated

Sorafenib 400 mg twice daily, every day, until progression or unacceptable side effects.

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00294 v4: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Sorafenib: differentiated thyroid cancer: treatment and checks

Progressive, locally advanced or metastatic differentiated thyroid cancer that no longer responds to radioactive iodine.

Sorafenib 400 mg twice daily, every day, until progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.	Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.	Keep a current dose and symptom record.	Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

Twice daily, continuously

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / **act:** Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, blood pressure, thyroid tests and assessment of heart function.

STEP 2

Taking or receiving treatment

Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.

STEP 3

Tests and reviews

Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check INR as indicated if taking warfarin.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Your exact treatment schedule

Tests and preparation

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your continuous-treatment review window.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Sorafenib every day	 **Days 1–28** Sorafenib 400 mg twice daily, every day, until progression or unacceptable side effects.

This 28-day window is a review calendar, not a rest cycle or a fixed treatment duration.

Treatment continues daily across review dates unless your team tells you to hold or stop. No automatic rest week is scheduled.

> Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Read the whole review window as text

Day 1–28: Every day, including weekends

Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself. Follow your prescription. A hold or restart must be agreed with the team.

Support around the medicines

1 Preventing and treating sickness

This is minimal-to-low oral sickness risk; the team agrees rescue medicine if needed.

2 Medicine-specific support

3 Mouth care, nutrition and daily life

Follow the unit's mouth-care plan. Report pain, poor intake or weight loss early; ask for dietetic support. Pace activity, rest and work around symptoms.

4 Medicine and interaction review

Bring all prescription, non-prescription and herbal medicines. Check with the oncology pharmacist before starting or stopping anything.

Your diagnosis and treatment setting

Specialist tests

Thyroid tests monitor hormone balance. Thyroglobulin and other response tests depend on the cancer subtype and specialist plan; they are not substitutes for scans.

Treatment aims and choices

Review and alternatives

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval. Discuss suitable alternatives, supportive care and the option of declining treatment.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Explain when you take or receive treatment.,Name the tests and next review date.,Show how to contact the team urgently.,Explain what to do if a dose is missed or treatment is held.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Sorafenib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.
- Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check international normalised ratio (blood-clotting test) (INR) as indicated if taking warfarin.
- Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

Sorafenib

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

Sorafenib

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Heart and blood pressure

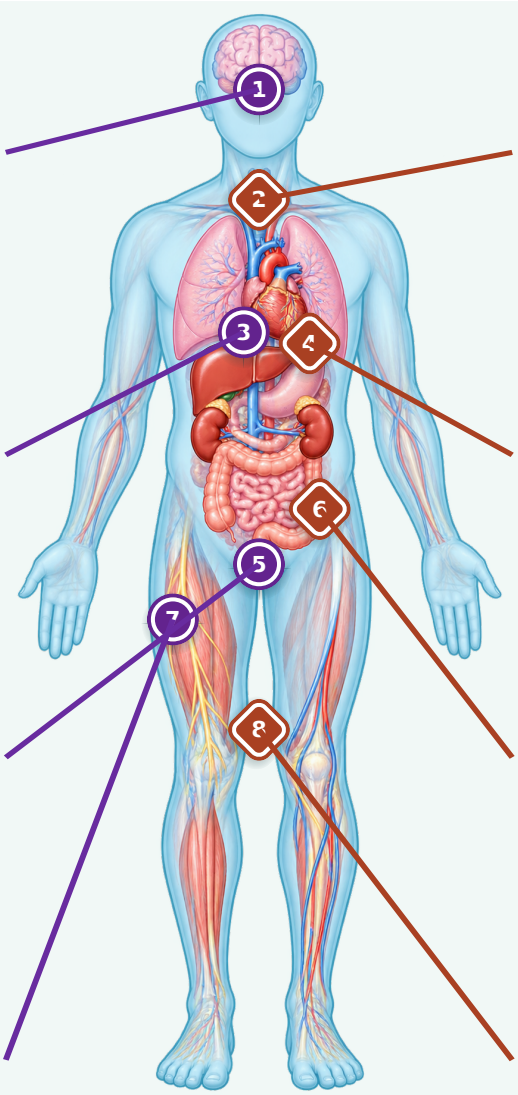
Sorafenib

Follow prescribed heart and blood-pressure checks. Chest pain, fainting, palpitations or new breathlessness needs urgent advice.

4 Breathing

Sorafenib

New cough or breathlessness needs prompt review; serious causes include infection or lung injury.



5 Sickness and bowels

Sorafenib

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

Sorafenib

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Skin, hands and feet

Sorafenib

Report painful palms or soles, rash or blistering early. Severe spreading rash or mouth/eye involvement needs urgent review.

8 Fertility and pregnancy

Sorafenib

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

One review per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record each review; boxes do not imply a fixed course.

Sorafenib · copy this sheet for the next review.

Sorafenib · Kinase inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Bowels	□ Usual pattern	□ New change: report	□ Persistent diarrhoea / constipation: call	□ Blood, severe pain or dehydration: urgent review	□ Collapse or severe sudden illness: 112/999
Skin	□ No change	□ New small change: report	□ Persistent rash / itch: contact team	□ Painful, spreading or blistering rash: urgent review	□ Severe skin reaction with acute illness: emergency help
Heart symptoms	□ None	□ New palpitations: contact team promptly	□ Palpitations with dizziness: urgent assessment	□ Chest pain or fainting: emergency assessment	□ Severe chest pain, collapse or breathing difficulty: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Differentiated thyroid cancer · NCCP 00294 v4 · Document revised 14 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Sorafenib

Twice daily, continuously

Phase: _____ Continuous treatment: team reviews dose

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My review record

Recording spaces only; not a fixed course.

Review	Date / initials

My treatment information

Latest review date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team records each review and the current daily dose. Recording boxes do not imply a fixed course or a rest period. Record any held dose or changed plan. An entry does not authorise a restart; keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.50 · NCCP 00294 v4 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00294 v4: official regimen**
   https://healthservice.hse.ie/documents/6704/294_v4_SORAfenib.pdf
2. **Nexavar: EMA product information, including trial evidence**
   https://www.ema.europa.eu/en/documents/product-information/nexavar-epar-product-information_en.pdf
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf